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Question 3

Not yet answered

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 [Flag question](#)

A 16-hour-old male neonate was presented to a hospital with a complaint of failure to suck of five hours duration. He was delivered in a hospital at gestational age of 35-week. On examination, he has grunting, respirations were 68/min, pulse rate was 178/min, and has pale skin color. On laboratory investigations, hemoglobin level was 5g/dl, and blood group is A+ and O+ for him and his mother respectively. What is the most likely cause of the neonate's condition?

- ☐ a. Rh incompatibility
- ☐ b. ABO incompatibility
- ☐ c. Vitamin K deficiency
- ☐ d. Anemia of prematurity

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Question 2

Not yet answered

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A 10-minute-old baby was delivered from a 30-year-old P-III mother with an assisted vacuum delivery. The baby's fifth minute APGAR score was 5 and is currently on resuscitation with bag and face mask ventilation. The heart rate is 55/min. What is the most appropriate next step in the management of the newborn?

- ☐ a. Continue to resuscitate with bag and face mask
- ☐ b. Stop resuscitation and give the baby to parents
- ☐ c. Intubation and Epinephrine
- ☐ d. Chest compressions coordinated with PPV

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Question 29

Not yet answered

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A 30-minute-old male neonate was presented with a complaint of fast breathing for 10 minutes. On physical examination, the neonate is in respiratory distress, respiration rate is 75/min, pulse rate is 154/min, temperature is 36.5 degree centigrade and gestational age is 43 weeks. Chest X-ray shows patchy infiltrates. What is the most likely diagnosis?

- ☐ a. Neonatal sepsis
- ☐ b. Birth asphyxia
- ☐ c. Meconium aspiration syndrome
- ☐ d. Hyaline membrane disease

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Question 74

Not yet answered

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 Flag question

A female neonate, who was delivered at gestational age of 43 weeks, has been admitted to a NICU for the last five days after being referred due to respiratory distress immediately after delivery. Physical examination findings at admission revealed over-distended chest and Meconium stained extremities. Currently, she has severe hypoxia with clear chest and good air entry. Otherwise, she has no detected abnormality. The chest radiograph is normal. What is the most likely diagnosis for the current problem?

- ☐ a. Spontaneous tension Pneumothorax
- ☐ b. Bacterial superimposed infection
- ☐ c. Congenital diaphragmatic hernia
- ☐ d. Persistent pulmonary hypertension

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Question 76

Not yet answered

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Flag question

A five-year-old female child with complaints of fever and rash of three days duration is brought to a hospital. She also has cough, nasal discharge and diarrhea. On physical examination, her temperature is 37.8 degree centigrade, respirations are 43/min, pulse rate is 123/min, and there is diffuse red maculo-papular rash from face to leg. What complication will be expected in her third decade of life?

- ☐ a. Pneumonia
- ☐ b. Severe malnutrition
- ☐ c. Subacute sclerosing panencephalitis
- ☐ d. Measles encephalitis

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Question 78

Not yet answered

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A four-year-old male child was brought by his mother to a clinic accompanied with a complaint of being unable to speak clear words. He clearly points to the thing he wants to have. He is able to ride tricycle and copies cross and circle on paper. Routine physical examinations did not show any abnormality. What is the most appropriate measure to be taken for the baby?

- ☐ a. Referring the child for mental retardation assessment
- ☐ b. Reassuring the mother to send him to regular school
- ☐ c. Reassuring the mother as it is normal
- ☐ d. Referring the child for hearing assessment

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Question 80

Not yet answered

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A mother has brought her three-month-old male infant to a clinic to get advice about his growth status. Anthropometry is done and all parameters are unaffected. What is the normal growth pattern to inform the woman?

- ☐ a. Head circumference increases by 2 cm per month for the next three months
- ☐ b. Weight increases by 30 g/day for the next three months
- ☐ c. Length increases by 0.5 cm per month for the next three months
- ☐ d. Weight doubles the birth weight at 4-5 months

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Question 81

Not yet answered

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A nine-hour-old male neonate was brought to a hospital with a complaint of failure to suck since birth. He was delivered at home and did not cry immediately after delivery. On physical examination, he has seizure on arrival; all neonatal reflexes are depressed, and lethargic. What is the first line treatment for this neonate?

- ☐ a. Phenytoin
- ☐ b. Phenobarbital
- ☐ c. Valproic acid
- ☐ d. Diazepam

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Question 86

Not yet answered

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A seven-month-old infant is brought to a hospital for a routine follow up. Growth and development of the infant are optimal. A WBC count of 7,000/mm³, hemoglobin level of 6.7g/dl with MCV of 60fl, and RDW of 20percent were noted from laboratory work up. What is the most likely diagnosis of the child?

- ☐ a. Thalassemia
- ☐ b. Lead poisoning
- ☐ c. Iron-deficiency anemia
- ☐ d. Folate deficiency anemia

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Question 87

Not yet answered

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A seven-year-old known cardiac patient, with atrioventricular septal defect and who is on follow up, is planning to undergo dental correction with manipulation of gingival tissue. He has no known history of drug allergy. What is the best first-line endocarditis preventive strategy for this child?

- ☐ a. Cefepime
- ☐ b. Amoxicillin
- ☐ c. Azithromycin
- ☐ d. Clindamycin

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Question 88

Not yet answered

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A six-week-old male infant was brought to a hospital with a complaint of rash for one hour. He took his first vaccine 12 hours back. On physical examination, his pulse rate is feeble, respirations are 65/min, he has stridor and wheezing over the chest, diffuse urticarial rash all over the body, and lip and oral mucosa swelling. What is the appropriate initial treatment for this infant?

- ☐ a. Blood transfusion
- ☐ b. Prednisolone
- ☐ c. Antihistamine
- ☐ d. Epinephrine

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Question 89

Not yet answered

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A three-year-old boy presented with inadequate intake and diarrhea of three days duration. His growth and development were optimal. On examination, skin pinch goes back very slowly and he is lethargic. His weight is 15kg. What is the most appropriate fluid therapy to this child?

- ☐ a. 1500ml RL over 24 hours
- ☐ b. 450ml RL over 30 minutes, followed by 1050ml over the next 2½ hours
- ☐ c. 450ml RL over 1 hour, followed by 1050ml over next 5 hours
- ☐ d. 300ml RL rapid bolus, followed by 450ml RL over 30minutes

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Question 90

Not yet answered

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A three-year-old female child was presented to a hospital with a complaint of fear of light (photophobia) for three months. She also has skin rash more on the arm, leg and buttocks areas. On physical examination, she has foamy appearing plaques on conjunctiva and dry, scaly, hyperkeratotic patches over the leg, arm and buttocks. What is the most likely diagnosis?

- ☐ a. Thiamine deficiency
- ☐ b. Vitamin A deficiency
- ☐ c. Zinc deficiency
- ☐ d. Riboflavin deficiency

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Question 91

Not yet answered

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 [Flag question](#)

A three-year-old male child was presented to a hospital with a complaint of fever for five days. He also has a yellowish discoloration of the eye and frequent abnormal body movement of one day duration. He is from malaria endemic area. On physical examination, icteric sclera, splenomegaly and hepatomegaly were found. What is the best first line treatment for this child?

- ☐ a. Artesunate
- ☐ b. Coartum
- ☐ c. Quinine
- ☐ d. Arthemeter

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Question 92

Not yet answered

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A three-year-old male child was presented to a hospital with a complaint of generalized body swelling of five days duration. On examination, he had puffy face, and his blood pressure was between the 50th and 90th percentile. On urinalysis, +3 protein was revealed. Cholesterol level of 400 mg/dl, triglyceride level of 250 mg/dl, serum creatinine level of 0.4 mg/dl, and BUN level of 15 mg/dl were noted on laboratory investigations. What is the most appropriate management for the child?

- ☐ a. Start Amoxicillin 40 mg/kg/day
- ☐ b. Start Furosemide 4 mg/kg/day
- ☐ c. Start Enalapril 200 microgram/kg/day
- ☐ d. Start Prednisolone 60 mg/m²/day

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Question 93

Not yet answered

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 [Flag question](#)

A two-hour-old male neonate was brought to a NICU after delivered from Hepatitis B surface antigen positive mother. On physical examination, his weight is 1500 gm; otherwise no other pertinent finding was detected. What is the most appropriate vaccination time of hepatitis B vaccine for this neonate?

- ☐ a. After weight reach 2.5kg
- ☐ b. At six weeks of age
- ☐ c. Within 12 hour of birth
- ☐ d. After one month of age

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Question 95

Not yet answered

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 [Flag question](#)

A two-month-old male infant presented with fast breathing and feeding interruption of two weeks duration. On examination, his pulse rate is 160/min, respirations are 60/min, temperature is 37.3 degree centigrade, oxygen saturation is 95percent, he has sub-costal and inter-costal retractions and total liver span is 9cm. What is the most likely diagnosis of this child?

- ☐ a. Pneumonia
- ☐ b. Perinatal tuberculosis
- ☐ c. Large VSD
- ☐ d. Severe anemia

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Question 96

Not yet answered

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 [Flag question](#)

A two-year-old female child presented to a hospital with a complaint offensive nasal discharge of three days duration. She also has a history of low-grade fever, cough and bad breath. Physical examination revealed bilateral erythematous and swollen nasal mucosa with purulent discharge. What is the most likely diagnosis in this child?

- ☐ a. Sinusitis
- ☐ b. Foreign body
- ☐ c. Lymphadenitis
- ☐ d. Periodontitis

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Question 94

Not yet answered

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A two-hour-old male neonate was brought to a NICU because he had fast breathing. He was delivered at 38 weeks via cesarean delivery with birth weight of 3500 gm and Apgar score of 8 and 10 in the first and fifth minutes respectively. Examination revealed a respiratory rate of 65/min in otherwise vigorous baby. What is the most likely diagnosis for this baby?

- ☐ a. Congenital pneumonia
- ☐ b. Pulmonary hypoplasia
- ☐ c. Transient tachypnea of the newborn
- ☐ d. Meconium aspiration syndrome

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Question 97

Not yet answered

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A two-year-old male toddler was presented to an EOPD with abnormal body movement which lasts ten minutes for the first time. On evaluation, the child started to convulse again. His RBS was 112 mg/dl. What is the most appropriate initial management?

- ☐ a. Phenobarbital 20 mg/kg PO
- ☐ b. Phosphenytoin 20 mg/kg IV
- ☐ c. Diazepam rectally 0.1ml/kg
- ☐ d. Dextrose 10percent 5ml/kg bolus

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